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Project #088: Increasing Access to Occupational and Physical Therapy for Postpartum Patients

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Abstract

Plan: Cesarean patients at Henry Ford Jackson rarely received OT/PT consults, unlike other surgical populations. Staff feedback revealed gaps in education and safe recovery practices. The goal was to improve access and equity by integrating rehab services into postpartum care.

Do: A phased rollout included stakeholder engagement, staff training, development of OB-specific competencies, and patient education tools. Consults were tracked, and advocacy for automated referrals was initiated.

Check: Therapy referrals increased steadily; 90% of referred patients were seen. Nurses began initiating consults. Of 264 cesarean deliveries, only 102 received therapy—highlighting the need for automated referrals. Outcome tracking remains limited but evolving.

Act: Next steps include implementing the Obstetrics Readiness for Hospital Discharge Scale, establishing OT/PT as standard of care for postpartum patients, and expanding the protocol system-wide.

Plan / Introduction

Problem Description: Occupational therapy (OT) and physical therapy (PT) post abdominal surgery are a standard of care throughout the rest of the hospital- however it was rare to receive consults post cesarean section. Our co-workers were coming back from maternity leave post cesarean section and endorsing that they were under educated in precautions and safe movement mechanics to prevent injury. We recognized an opportunity to address this gap by identifying barriers to safe discharge and enhancing education for new mothers—particularly those recovering from cesarean section. Through the specialized services of our Rehabilitation Department, including both occupational and physical therapy, we can provide targeted interventions that promote safe mobility, support functional recovery, and equip new moms with the skills and knowledge they need for a confident transition home.

Available Knowledge: Postpartum rehabilitation services like occupational and physical therapy are rarely included in standard maternal care, especially after cesarean delivery. This gap disproportionately affects marginalized women, contributing to preventable complications and inequitable recovery outcomes.

System-Level Barriers

- Lack of Standardized Clinical Pathways – Few hospitals have formal protocols for initiating occupational or physical therapy after childbirth, unlike other surgical populations
- Weak Integration into Obstetric Care – Rehab services are often siloed from maternity teams, leading to missed opportunities for early intervention

Provider-Related Barriers

- Low Awareness of Rehab Benefits – Many obstetric and nursing staff are unfamiliar with the evidence supporting early PT/OT for postpartum recovery
- Bias in Referral Patterns – Implicit bias and systemic inequities can result in lower referral rates for patients from marginalized racial, ethnic, or socioeconomic groups and perhaps just from maternal health diagnosis perspective

Bottom line: The absence of structured, reimbursable, and integrated postpartum rehabilitation—combined with systemic inequities—creates a significant healthcare gap. Addressing these barriers requires policy change, provider education, and embedding PT/OT into standard obstetric care pathways to ensure equitable recovery support for all mothers.^{1, 3, 4}

Rationale:

The study Reducing Maternal Morbidity on the Frontline: Acute Care Physical Therapy After Cesarean Section During and Beyond the COVID-19 Pandemic proposes adapting the Abdominal Core Surgery Rehabilitation Protocol—originally designed for post-hernia repair patients—as a framework for cesarean recovery. This approach emphasizes activity modifications to reduce abdominal pressure, pain, and surgical wound tension, aiming to improve postpartum outcomes through targeted physical therapy interventions.¹

Building on the Abdominal Core Surgery Rehabilitation Protocol and incorporating principles from the Enhanced Recovery After Delivery™ framework, we developed our current standard of care protocol to address the unique rehabilitation needs of postpartum patients following cesarean section. By developing a tailored competency for rehab professionals, this approach ensures early, evidence-based care within 24–48 hours post-delivery, targeting pain reduction, abdominal pressure management, and improved functional recovery.²

Specific Aim(s)/Goal(s): To address inequitable access to inpatient rehabilitation services for postpartum patients following cesarean delivery

- **S (Specific):** Expand post-operative occupational and physical therapy services for patients after cesarean section
- **M (Measurable):** Track progress through increased therapy consults initiated by the acute care team
- **A (Achievable):** Ensure that at least 75% of consults result in patients being evaluated by therapy staff
- **R (Relevant):** Bridge clinical knowledge gaps and enhance quality of care by improving access to early rehabilitation
- **T (Time-bound):** Achieve these outcomes by August 2025

Do / Methods

Programming Timeline and Progress

Phase 1: Stakeholder Engagement & Initial Buy-In: Build foundational support across departments

Week 1–2: Met with unit director and nursing manager to introduce the initiative and engaged obstetricians to secure clinical support

Week 2-3: Presented to the Obstetrics Perinatal Collaborative and rehab teams from other hospitals to align goals and share best practices

Phase 2: Unit-Level Integration & Staff Mobilization/Training: Establish frontline awareness and workflow readiness

Week 4-5: Met with unit educator to discuss training needs and implementation logistics. Attended morning and evening RN huddles to build floor-level support and encourage consults. Distributed laminated reference cards detailing therapy order procedures to unit staff

- Competency Development & Staff Training: Developed OB floor therapy competency for OT/PT staff. Conducted training sessions to ensure therapists are prepared to meet the unique needs of postpartum patients

Week 6: Nurses began screening and initiating OT/PT orders for postpartum patients

Phase 3: System Advocacy & Expansion Efforts: Advocate for systemic change and broader adoption

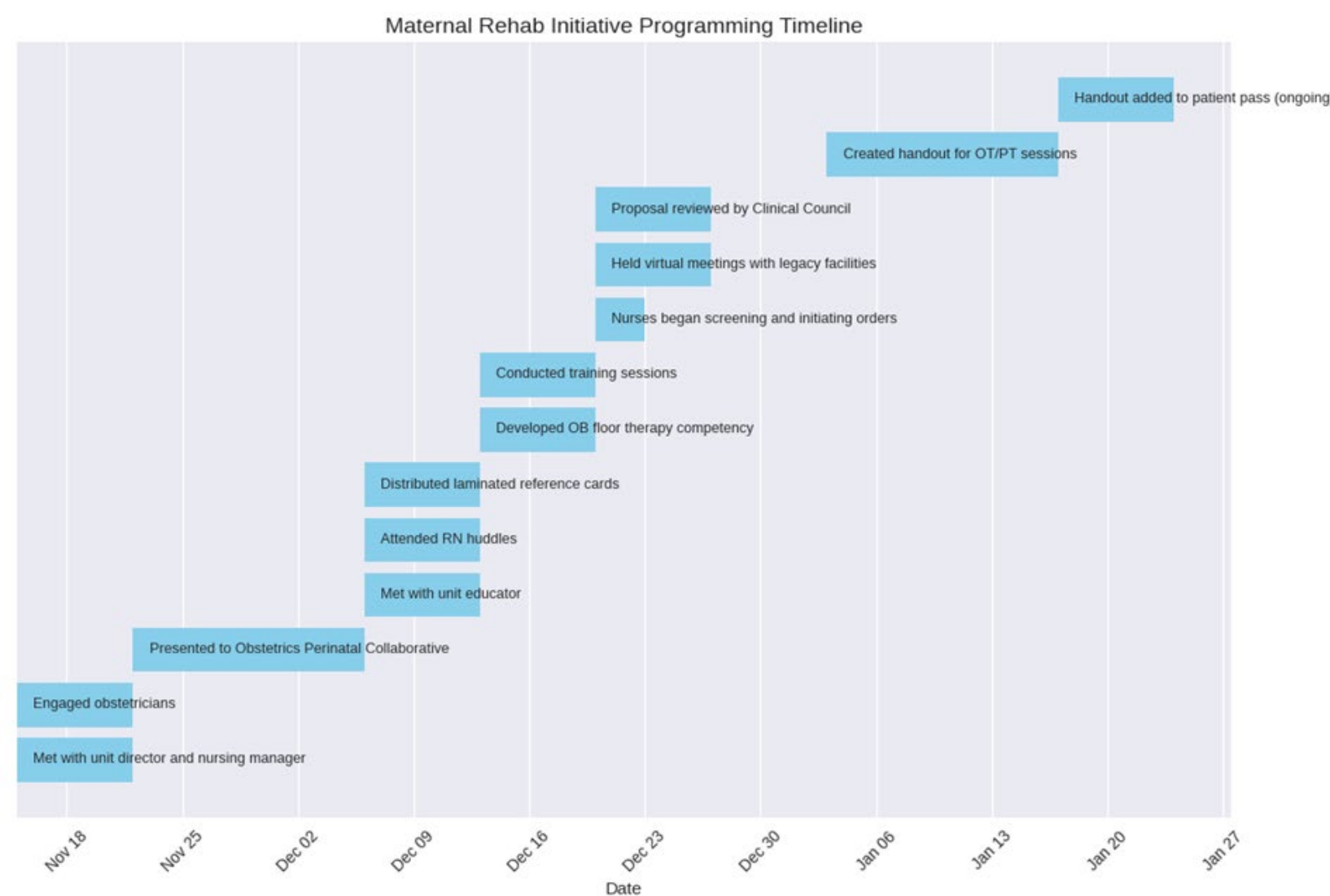
Week 6-7: Held virtual meetings with legacy Henry Ford facilities to propose automatic therapy orders post-cesarean. Proposal reviewed by Women and Children’s Clinical Council

Outcome: Automatic orders not approved due to readiness gaps at other therapy sites, despite strong support from select stakeholders

Phase 4: Patient-Facing Tools & Quality Enhancement: Improve patient education and continuity of care

Week 8-10: Created a handout for use during OT/PT sessions to reinforce recovery strategies

Ongoing: Handout added to patient pass post-session to support continued engagement and understanding along with evaluation/assessments when in acute care setting



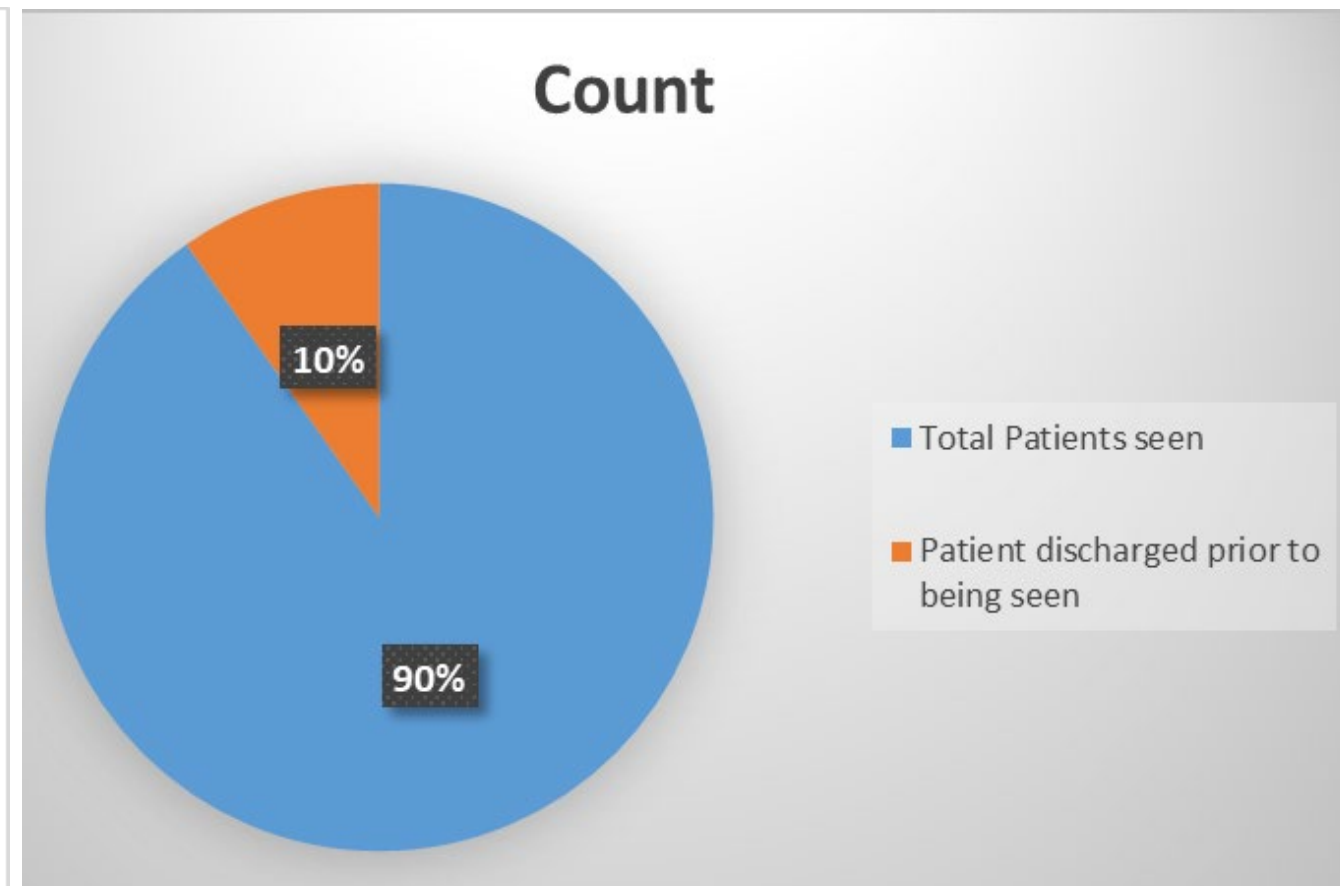
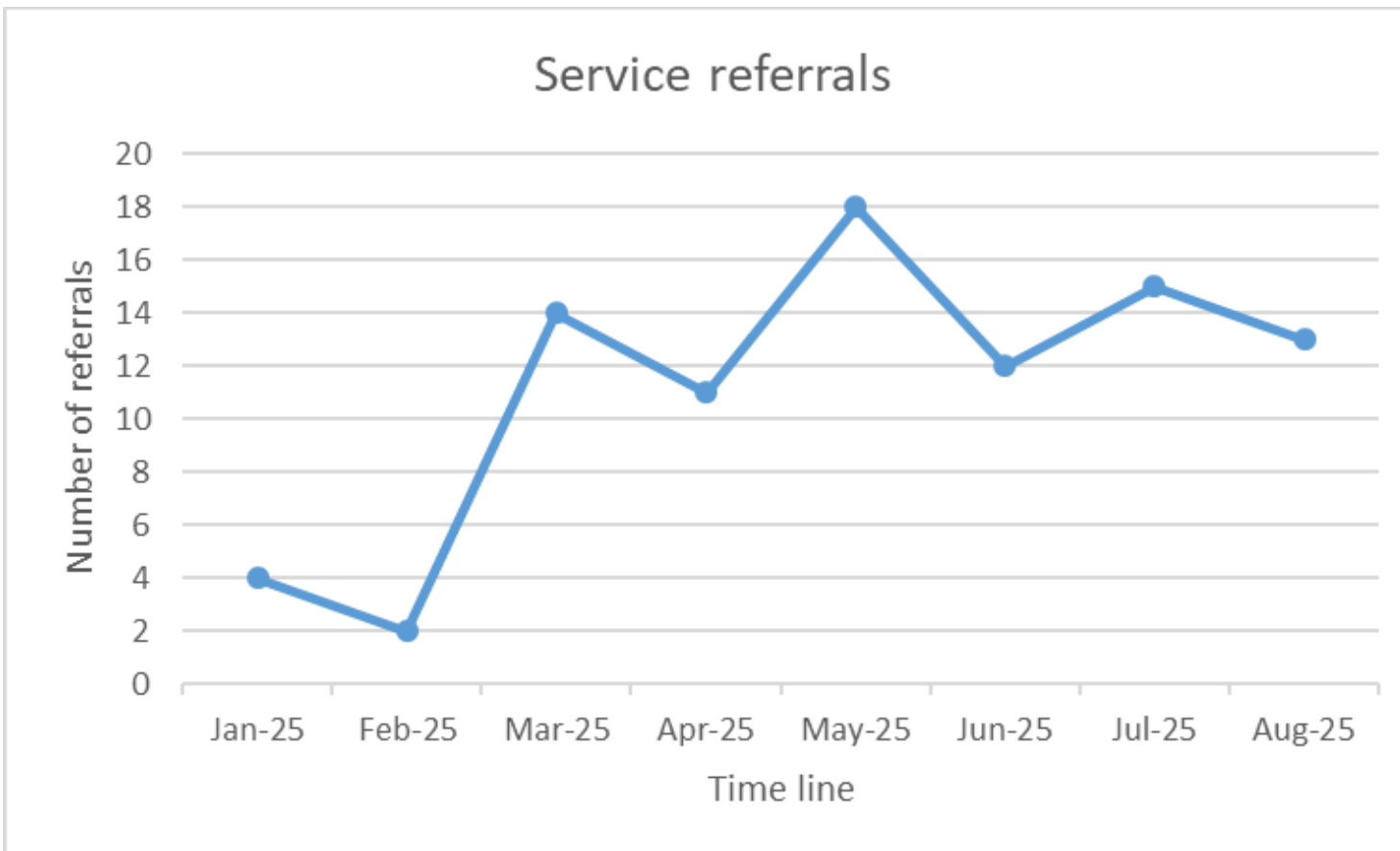
Measures & Analysis: To evaluate the effectiveness of the intervention, a multi-pronged tracking and outcome validation strategy was employed:

Consult Tracking via Excel: Therapy consults for post-cesarean patients were monitored using a dedicated excel sheet. The goal was to ensure that at least 75% of cesarean patients were offered occupational and/or physical therapy services. This allowed for real-time visibility into consult rates and service delivery.

Referral Trends Over Time: Over the 8-month trial period, referrals steadily increased, indicating growing provider awareness and buy-in. This upward trend continues, suggesting sustained engagement and perceived value of therapy services.

Service Delivery Rate: Of all referrals received, 90% of patients were successfully seen by therapy staff. The remaining 10% were discharged prior to service due to timing or medical factors, highlighting logistical challenges rather than lack of clinical intent.

Future Objective Measures: To strengthen outcome attribution, the Hospital Readiness for Discharge Scale will be implemented pre- and post-therapy. This validated tool will help quantify functional gains and readiness improvements directly linked to therapy interventions.



Check / Results

The project began with strategic stakeholder engagement, including leadership, obstetricians, and collaborative partners, to build foundational support. Unit-level integration followed, with staff training, RN huddle participation, and distribution of therapy ordering guides to increase consults. A specialized OB therapy competency was developed and implemented for rehab staff. Nurses began initiating OT/PT orders by Week 6, marking a shift toward frontline ownership. System-level advocacy for automatic therapy orders was pursued but not approved due to readiness gaps across sites. To enhance patient education, a recovery handout was created and incorporated into therapy sessions and patient passes. The intervention evolved through continuous feedback and adaptation, laying the groundwork for sustainable practice change.

Act / Discussion

This initiative addressed the lack of standardized postpartum rehabilitation following cesarean delivery, advancing access and equity in care. Over eight months, therapy consults steadily increased, with 90% of referred patients successfully evaluated. Nurse-initiated consults and integrated patient education tools signaled strong frontline adoption.

Strengths

- Early stakeholder engagement and cross-disciplinary collaboration
- Tailored OB therapy competency for rehab staff
- Real-time consult tracking and feedback
- Use of evidence-based frameworks (e.g., ERAD™, Abdominal Core Surgery Rehabilitation Protocol)

Interpretation: Staff training and workflow integration were strongly associated with increased therapy access. Results align with existing literature on maternal morbidity, reinforcing the need for systemic change. However, automatic therapy orders were not approved due to readiness gaps, revealing institutional variability and the challenge of scaling innovation across systems. Of 264 cesarean deliveries, only 102 referrals were placed—highlighting a critical gap and supporting the case for automated referrals.

Limitations: Outcome data relied on consult tracking rather than standardized functional measures, pending implementation of the Hospital Readiness for Discharge Scale.

Conclusion: Integrating PT/OT into postpartum care is feasible, impactful, and well-received. With continued refinement, the model shows promise for broader adoption and long-term sustainability.

Next steps:

- **Implement Standardized Outcome Measurement:** Begin using the Obstetrics Readiness for Hospital Discharge Scale to assess functional progress and discharge readiness for postpartum patients.
- **Establish OT/PT as Standard of Care:** Ensure occupational and physical therapy consults are consistently provided to all birthing mothers and women’s health patients post-surgery at Henry Ford Jackson Hospital
- **Expand System-Wide Adoption:** Scale the protocol across the entire Henry Ford Health system, making OT/PT consults a standard of care for postpartum and surgical recovery in women’s health.

Ethical Considerations:

- **Empowering Patient Choice Through Education:** By embedding therapy consults into routine care, patients are better informed about their recovery options
- **Optimizing Resource Use for Better Outcomes:** While expanding services may increase consult volume, it also creates opportunities to streamline care, reduce complications, and prevent readmissions. Strategic implementation ensures that resources are used efficiently to support long-term maternal wellness
- **Promoting Equitable Access to Care:** Standardizing OT/PT consults for postpartum patients ensures that all birthing mothers—regardless of background or diagnosis—receive consistent, evidence-based support during recovery. This helps close gaps in care and advances health equity in maternal services

Other Information

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